# Week 9 Questions

# Electrolytes

A client with chronic heart failure presents with confusion, nausea, and a sodium level of 128 mEq/L. Which cause most likely explains this finding?

A. Excessive sweating
B. Fluid retention
C. Inadequate dietary intake of salt
D. Renal potassium wasting

# Which client is at risk for development of a sodium level of 130 mEq/L?

A. The client who is taking diuretics
B. The client with hyperaldosteronism
C. The client with Cushing’s syndrome
D. The client who is taking corticosteroids

# A client with SIADH has a serum sodium of 118 mEq/L. The nurse finds the client newly confused and lethargic. Which action should the nurse take first?

A. Obtain a urine specific gravity
B. Initiate seizure precautions and notify the provider immediately
C. Reorient the client and reassess in 2 hours
D. Encourage the client to drink more water

# A client with a sodium of 150 mEq/L is treated with IV fluids. Which principle is most important?

A. Decrease sodium level rapidly
B. Increase dietary sodium
C. Decrease sodium level slowly
D. Administer diuretics immediately

# A client has K of 6.1 mEq/L. Which provider order should the nurse complete first?

A. Administer oral potassium
B. Place the client on a cardiac monitor
C. Start potassium-sparing diuretic
D. Encourage high potassium foods

# Potassium chloride intravenously is prescribed for a client with heart failure experiencing hypokalemia. Which actions would the nurse take to plan for preparation and administration of potassium? Select all that apply.

A. Obtain an IV pump
B. Monitor urine output during administration
C. Prepare the medication for bolus administration
D. Monitor the IV site for signs of infiltration or phlebitis
E. Ensure the medication is diluted in the appropriate volume of fluid
F. Ensure the bag is labeled with the volume of potassium in the solution

# A client with severe hypokalemia (K 2.6) has a new order for potassium chloride IV. Which nursing action is the priority?

A. Check the client’s urine output at the end of the shift
B. Administer the potassium as an undiluted IV push for rapid correction
C. Verify the dilution and rate, use an infusion pump, and place the client on cardiac monitoring
D. Encourage the client to eat potassium rich foods

# A client on furosemide (Lasix) develops K of 3.0 mEq/L. Which intervention is appropriate?

A. Give IV potassium as a bolus push
B. Discontinue all fluids
C. Administer oral potassium supplements
D. Restrict potassium rich foods

# Which of the following statements are correct about hypocalcemia? Select all that apply.

A. It can lead to osteopenia
B. Dialysis is always required as a treatment
C. Inadequate vitamin D can lead to hypocalcemia
D. Puts client at risk for seizures

# Which intervention is appropriate for mild hypocalcemia?

A. Administer loop diuretics
B. Administer kayexalate
C. Restrict fluid intake
D. Increase intake of vitamin D and calcium-rich foods

# A client is 1 day post total thyroidectomy. The client repots numbness and tingling around the mouth and in the fingertips. Which action should the nurse take first?

A. Notify the surgeon of the symptoms
B. Reassure the client that this is a normal postoperative finding
C. Document the findings in the chart
D. Assess for a positive Chvostek’s or Trousseau’s sign

# Which complication can occur if IV magnesium is infused too rapidly?

A. Tachycardia
B. Respiratory depression
C. Hyperreflexia
D. Hypertension

# The nurse is caring for four clients on a medical-surgical unit. Which client should the nurse assess first?

A. A client with hyperkalemia (K 6.8) with peaked T waves on the monitor
B. A client with hypercalcemia (Ca 11) who reports constipation
C. A client with hyponatremia (Na 131) who is mildly fatigued
D. A client with hypokalemia (K 3.2) who reports leg cramps

# Fluid Imbalances

# On review of the clients' medical records, the nurse determines that which client is at risk for fluid volume excess?

A. The client taking diuretics who has tenting of the skin
B. The client with an ileostomy from a recent abdominal surgery
C. The client who requires intermittent gastrointestinal suctioning
D. The client with kidney disease that developed as a complication of diabetes mellitus

# The nurse caring for a client with heart failure who has been receiving IV diuretics suspects that the client is experiencing a fluid volume deficit. Which assessment finding would the nurse note in a client with this condition?

A. Weight loss and poor skin turgor
B. Lung congestion and increased heart rate
C. Decreased hematocrit and increased urine output
D. Increased respirations and increased blood pressure

# The nurse is assigned to care for a group of clients. On review of the client's medical records, the nurse determines that which client is most likely at risk for a fluid volume deficit?

A. A client with an ileostomy
B. A client with heart failure
C. A client on long-term corticosteroid therapy
D. A client receiving frequent wound irrigations

# The nurse is caring for a client with severe dehydration from vomiting and diarrhea. Vital signs: BP 84/50, HR 124, RR 22. Which prescribed intervention should the nurse implement first?

A. Obtain a 24 hour diet recall
B. Insert a urinary catheter for strict intake and output
C. Start isotonic IV fluid bolus
D. Provide oral care and sips of clear liquids

# ABGs

# The nurse is caring for a client with several broken ribs. The client is most likely to experience which type of acid-base imbalance?

A. Respiratory acidosis from inadequate ventilation
B. Respiratory alkalosis from anxiety and hyperventilation
C. Metabolic acidosis from calcium loss due to broken bones
D. Metabolic alkalosis from taking analgesics containing base products

The nurse is caring for a client who is on a mechanical ventilator. Blood gas results indicate a pH of 7.50 and PaCO2 of 30. The nurse has determined that the client is experiencing respiratory alkalosis. Which laboratory value would most likely be noted in this condition?

A. Magnesium level of 1.8
B. Sodium level of 145
C. Potassium level of 3.0
D. Magnesium level of 2.0

The nurse reviews the blood gas results of a client with atelectasis. The nurse analyzes the results and determines that the client is experiencing respiratory acidosis. Which result validates the nurse's findings?

A. pH 7.25, $PaCO_2$ 50
B. pH 7.35, $PaCO_2$ 40
C. pH 7.50, $PaCO_2$ 52
D. pH 7.52, $PaCO_2$ 28

The nurse is caring for a client with a nasogastric tube that is attached to low suction. The nurse monitors the client for manifestations of which disorder that the client is at risk for?

A. Metabolic acidosis
B. Metabolic alkalosis
C. Respiratory acidosis
D. Respiratory alkalosis

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All but C

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A, C, D

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